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## Pathology Report:

## Surgical Pathology Report

### FINAL PATHOLOGIC DIAGNOSIS

Pelvic exenteration and lymph node dissection;

#### A. Scar tissue-abdomen:

- Skin and subcutaneous tissue with fibrosis, no tumor present.

#### B. Anterior pelvic exenteration:

- Urothelial carcinoma of the urinary bladder, high grade, with extensive squamous differentiation, involving left intramural ureter and transmurally invasive into anterior cervical wall and vagina.
- Urothelial carcinoma in situ (flat urothelial carcinoma).
- Extensive perineural invasion
- No unequivocal lymphovascular invasion present.
- Margins of resection (soft tissue, urethra, right and left ureter), no tumor present.

- See pathologic parameters and comment.

- Uterus with atrophic endometrium and leiomyoma (0.6 cm), no tumor present.

- Left fallopian tube with paratubal cysts, no tumor present.

- Bilateral ovaries and right fallopian tube with no significant pathology,

no tumor present.

#### C. Left ureter:

- Segment of ureter, no tumor present.

#### D. Right distal ureter:

- Segment of ureter, no tumor present.

#### E. Right hypogastric node:

- Three lymph nodes, no tumor present (0/3).

#### F. External iliac:

- Three lymph nodes, no tumor present (0/3).

#### G. Presacral node:

- One lymph node, no tumor present (0/1).

#### H. Common iliac:

- One lymph node, no tumor present (0/1).

#### I. Paracava:

- One lymph node, no tumor present (0/1).

#### J. Left obturator:

- One lymph node, no tumor present (0/1).

#### K. Left common iliac:

- Four lymph nodes, no tumor present (0/4).

#### L. Left hypogastric:

- Three lymph nodes, no tumor present (0/3).

#### M. Left common node:

- Two lymph nodes, no tumor present (0/2).

#### N. Distal right ureter:

- Segment of ureter, no tumor present.

#### O. Right obturator lymph node:

- Eight lymph nodes, no tumor present (0/8).

#### P. Round ligament:

- Dense fibromuscular tissue, no tumor present.

## Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Urothelial carcinoma with extensive squamous differentiation
2. Grade of tumor: High grade

| Criteria                       | Yes       | No                                  |
|--------------------------------|-----------|-------------------------------------|
| Diagnosis Discrepancy          |           | <input checked="" type="checkbox"/> |
| Primary Tumor Site Discrepancy |           | <input checked="" type="checkbox"/> |
| HIPAA Discrepancy              |           | <input checked="" type="checkbox"/> |
| Prior Malignancy History       |           | <input checked="" type="checkbox"/> |
| Dual/Synchronous Primary Noted |           | <input checked="" type="checkbox"/> |
| Case Is (Circle):              | QUALIFIED | DISQUALIFIED                        |
| Reviewer Initials              | 10/21/11  | 9/22/11                             |

1CD-0-3

carcinoma, urothelial, NOS 8/20/3

Path. Site: bladder, wall, NOS C67.9

CQCF: bladder, wall, lateral C67.2

hw  
10/21/11

3. Depth of invasion: Extravesical adipose tissue, anterior cervical wall and vagina
4. Tumor distribution: Solitary (8.5 x 7 x 3.5 cm)
  - Right lateral wall
  - Left lateral wall
  - Anterior wall
  - Posterior wall
5. Ureteral margins: Negative
6. Distal urethral margin: Negative
7. Soft tissue margin or serosa: Negative
8. Lymph nodes: Twenty seven lymph nodes, no tumor present (0/27)
9. pTNM: pT4a, N0, Mx

Effective this Checklist utilizes the 7th edition TNM staging system for Bladder on the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

#### Comment

The entire invasive portion of the tumor consists of keratinizing squamous carcinoma. The in-situ component shows both urothelial and squamous cell carcinoma differentiation (slide labeled B11). Therefore, the tumor is considered urothelial carcinoma with extensive squamous differentiation.

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[], M.D

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [] M.D.

Electronically Signed Out by [] M.D

#### Clinical History:

The patient is a -year-old female with a history of bladder cancer who undergoes pelvic exenteration.

#### Specimens Received:

- A: Scar tissue-abdomen
- B: Anterior pelvic exenteration
- C: Left ureter
- D: Right distal ureter
- E: Right hypogastric node
- F: External iliac
- G: Presacral node
- H: Common iliac
- I: Paracava
- J: Left obturator
- K: Left common iliac
- L: Left hypogastric
- M: Left common node
- N: Distal right ureter
- O: Right obturator lymph node
- P: Round ligament

#### Gross Description:

The specimens are received in 16 containers each labeled with the patient's name and medical record number.

Part A. is no additional designation. Received fresh and placed in formalin are 2 elliptical portions of wrinkled brown skin. One is 7.1 x 2.3 cm, excised to a maximum depth of 0.3 cm, and the second is 6.2 x 2 cm, excised to a maximum

depth of 0.4 cm. Sectioning reveals no masses or lesions. A representative section of each piece is submitted in A1.

Part B. is further labeled "2. Anterior pelvic exenteration". Received fresh and placed in formalin is a specimen consisting of bladder and uterus which is overall 20 x 11 x 5.5 cm. The bladder is 10 x 11 x 5.5 cm and covered with lobulated yellow fibroadipose tissue with cautery artifact. The anterior wall and around the urethral margin is inked blue. There is a small amount of apparent labial tissue adjacent to the urethral margin. The labium is 5 x 3 x 0.4 cm and wrinkled gray-brown and unremarkable. The right ureter is 2 cm long and 2.5 cm in diameter. The left ureter is 0.6 cm long and 0.6 cm in diameter. After shaving the urethral margin, the bladder is opened along the anterior with a Y-shaped incision, revealing a partially ulcerated, partially fungating mass along the posterior and left wall, extending across the anterior partially into the right lateral wall. The overall dimensions are 8.5 x 7 cm, with a raised portion up to 1.8 cm above the mucosa. The mass comes within 4 cm of the urethral margin. Sectioning through the left ureter reveals that the mass encases the ureter and extends to the blue inked margin. Sectioning through the posterior wall reveals that the tumor extends to the cervix, with a maximum depth of 3.5 cm. The tumor also extends into the right wall and comes within 0.1 cm of the inked margin. The uninvolved mucosa is edematous. No lymph node candidates are identified in the attached adipose tissue. Gross photographs are taken.

Posterior to the bladder is a 9 x 6 cm portion of wrinkled pink-tan mucosa consistent with vaginal wall at terminating in a cervical opening. The anterior half of the cervix is a 1.3 by the 1.3 and apparent posterior half is missing and consists of cauterized tissue. The attached uterus is 5 cm from cornu to cornu 4.8 cm from fundus to lower uterine segment and 3.3 cm from anterior to posterior. The serosa covering the uterus is glistening red and unremarkable. The left fallopian tube has a white clip. The left fallopian tube is 8 cm long and up to 1.2 cm in diameter. The left ovary is 2.5 x 1.8 x 0.9 cm. The right fallopian tube has no clip but is focally disrupted. The length of the distal portion is 7 cm with a circumference of 0.7 cm. The right ovary is 2.5 x 1.5 x 0.7 cm. The bilateral ovaries are bisected to reveal unremarkable gray firm tissue. The uterus is opened from anterior to posterior to reveal a 3.4 x 0.3 by 0.3 cm endometrial cavity with unremarkable tan lining. No masses or lesions are identified in the uterus. Sectioning through the anterior and posterior endomyometrium reveals an endometrial thickness of less than 0.1 cm. The posterior endomyometrium has a maximum thickness of 1.4 cm, and the anterior endomyometrium has a maximum thickness of 2 cm. There is a single white whorled nodule in the anterior myometrium which is 0.6 cm in greatest diameter. The endocervical canal is 1.8 cm long with unremarkable tan trabeculated mucosa.

Representative sections are submitted as follows:

- B1: Urethral margin
- B2: Left ureter margin
- B3: Right ureter margin
- B4: Labia D5 closest approach to urethral margin
- B5: Approach of tumor to urethral margin
- B6 and B7 section through left ureter, bisected
- B8 and B9: Section through ureter and anterior cervix, bisected
- B 10: Anterior wall tumor
- B. 11: Tumor in the right wall
- B12: Uninvolved dome of bladder
- B. 13: Posterior cervix
- B. 14: Anterior endomyometrium including whorled nodule
- B. 15 posterior endomyometrium
- B. 16: Right ovary and fallopian tube
- B-17 left ovary and fallopian tube

Part C. is further designated all 3. Left ureter". Received fresh for

intraoperative diagnosis is a 1 cm long specimen with a staple approximately in the middle. The specimen is bisected and both halves are frozen. The frozen section diagnosis is low benign ureter (both ends)" per Dr.[] The frozen section remnant and entirely submitted in C1.

Part D. is further designated "4. Right distal ureter". Received fresh for intraoperative diagnosis is a 1 cm long portion of ureter with a staple in approximately the middle. The specimen is bisected and both halves are frozen. The frozen section diagnosis is "benign ureter (both ends)" per Dr. []. The frozen section remnant is entirely submitted in D1.

Part E. is further designated "5. Right hypogastric node". Received fresh and placed in formalin is a 2.5 x 2 x 1.3 cm aggregate of lobulated fibrofatty tissue. Sectioning reveals 2 lymph node candidates ranging from 1.5 to 2 cm in greatest dimension. Each candidate is bisected and the specimen is entirely submitted as follows:

- E1: one lymph node candidate
- E2: One lymph node candidate
- E3: remaining fibrofatty tissue

Part F. is further designated "6. External iliac". Received fresh and placed in formalin is a 5 x 3 x 1.5 cm aggregate of lobulated yellow tan fibrofatty tissue. Sectioning for lymph nodes reveals one lymph node candidates ranging from 1.5-1.6 cm in greatest dimension. The specimen is entirely submitted as follows:

- F1: One lymph node candidate
- F2: One lymph node candidate, bisected
- F3: One lymph node candidate, bisected
- F4: One lymph node candidate, bisected
- F5 F6: remaining fibrofatty tissue

Part G. is further designated "7. Pre-sacral node". Received fresh and placed in formalin is a 2.8 x 2 x 1 cm aggregate of lobulated fibroadipose tissue. Sectioning reveals no obvious lymph node candidates. Specimen is entirely submitted in G1 and G2

Part H is further designated "8. Common iliac". Received fresh and placed in formalin is a 2.6 I 1.8 x 0.3 cm aggregate of lobulated yellow-tan fibroadipose tissue. Sectioning reveals a single lymph node candidate measuring 1.5 cm in greatest diameter. The specimen is entirely submitted as follows:

- H1: Lymph node candidate, bisected
- H2: remaining fatty tissue

Part I is further designated "9.Paracaval". Received fresh and placed in formalin is a 1.5 x 1.2 x 0.6 cm aggregate of lobulated fibrofatty tissue with a 0.5 cm in greatest lymph node candidate. The specimen is entirely submitted in I1.

Part J. is further designated "10. Left obturator" Received fresh and placed in formalin is a 3.4 x 2.9 x 1.3 cm aggregate of a relatively firm fibrous tissue with lobulated yellow adipose tissue. Individual lymph nodes cannot be identified. The specimen is sectioned and entirely submitted in J1-J5.

Part K. is further designated "11 left common iliac.". Received fresh and placed in formalin is a 3.5 x 3 x 1.5 cm aggregate of lobulated yellow-tan fibrofatty tissue. Dissection for lymph nodes reveals 3 lymph node candidate ranging from 1-2.2 cm in greatest diameter. The specimen is entirely submitted as follows:

- K-1: One lymph node tissue, bisected
- K2: One lymph node candidate, bisected
- K3 and K4: One lymph node candidate, bisected

K5 and K6: Remaining fibrofatty tissue

Part L. is further designated "12. Left hypogastric". Received fresh and placed in formalin is a 2.6 x 1.7 x 1.7 cm aggregate of fibrofatty tissue. Dissection for lymph nodes reveals 2 lymph node candidates ranging from 0.5-1.1 cm in greatest diameter. The specimen is entirely submitted as follows:

L1: 1 lymph node candidate

L2: One lymph node candidate, bisected

L3 and L4: Remaining fibrofatty tissue

Part M. is further designated "13. Left common node". Received fresh and placed in formalin is a 3.5 x 1.8 x 1.5 cm aggregate of fibrofatty tissue. Sectioning reveals one lymph node candidate, 5 cm in greatest dimension. Specimen is entirely submitted as follows:

M1 - M3: One lymph node candidate, section

M4 - 6: Remaining fibrofatty tissue

Part N is further designated "14. Distal right ureter". Received fresh and placed in formalin is a 1.7 cm long by 1 cm in diameter portion of apparent ureter. There is a stitch at one end which is not designated. This end is taken to be not to the margin is inked black. The end with the presumed margin is inked blue. The specimen is opened lengthwise and sectioned and entirely submitted in N1 and N2.

Part O. is further designated "3. Right obturator lymph node". Received fresh and placed in formalin is a 4 x 2.5 x 1.5 cm aggregate of lobulated yellow-tan fibrofatty tissue. Sectioning for lymph node candidates reveals 5 lymph node candidates ranging from 0.8-2.1 cm in greatest diameter. The specimen is entirely submitted as follows:

O1: 3 lymph node candidates

O2: One lymph node candidate

O3-O4: One lymph node candidate, sectioned

O5 and O6: Remaining fibrofatty tissue

Part P is further designated "16 round ligament". Received fresh and placed in formalin are 2 portions of fibroadipose tissue ranging from 1.7 x 1 x 0.3 cm to 0.9 x 0.7 x 0.3 cm. The specimen is entirely submitted in P1.

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[], M.D., Ph.D